AETNA CLINICAL POLICY BULLETIN

CPB 0596 — Liver Transplantation (Adult Orthotopic)

Effective 2026-01-01 · Last reviewed 2026-03

§I. Scope

Aetna covers adult orthotopic liver transplantation for medically necessary indications when performed at a UNOS-approved transplant center. Living-donor and deceased-donor procedures are both in scope. This policy aligns with OPTN policy 9 and AASLD 2024 transplant guidance.

§II. Medical necessity criteria (any qualifies)

A. End-stage liver disease (ESLD) with one or more:
1. MELD-Na score ≥ 15
2. Decompensated cirrhosis (variceal bleed, ascites, hepatic encephalopathy, spontaneous bacterial peritonitis)
3. Refractory hepatic hydrothorax

B. Hepatocellular carcinoma (HCC) meeting Milan criteria (single tumor ≤ 5 cm OR up to 3 tumors each ≤ 3 cm, no extrahepatic spread, no major vascular invasion).
C. Acute liver failure (King's College or Clichy criteria met).
D. Genetic / metabolic indications — Wilson's disease, hereditary hemochromatosis, α1-antitrypsin deficiency, primary hyperoxaluria.

§III. Required pre-transplant workup

1. MELD-Na within 90 days of listing.
2. Cardiac clearance (stress echo or coronary angiography per age/risk).
3. Pulmonary clearance (PFTs, room-air ABG; rule out hepatopulmonary syndrome and porto-pulmonary hypertension).
4. Renal function assessment (eGFR, urinalysis).
5. Infectious screening: HBV, HCV, HIV, CMV, EBV, VZV, syphilis, latent TB.
6. Cancer screening per age/risk.
7. Psychosocial evaluation by transplant social worker.
8. **Substance use:** documented abstinence per program criteria (commonly ≥ 6 months for alcohol; varies by center). 6-month rule may be waived for severe alcohol-associated hepatitis with multidisciplinary review per AASLD 2018 recommendations.

§IV. Allocation principles

MELD-Na is the primary allocation score. Standardized exception points are applied for HCC (MMaT-3), pulmonary complications, and selected metabolic/cholestatic conditions per OPTN policy 9.

§V. Post-transplant immunosuppression

Standard initial regimen: tacrolimus + mycophenolate ± corticosteroids. mTOR inhibitors (everolimus, sirolimus) considered for HCC indication or renal-sparing. Drug-drug interactions with HCV DAAs must be screened (see CPB 0860).

§VI. References

OPTN policy 9 · AASLD 2024 transplant guidance · MELD-Na allocation (NEJM 2008) · Milan criteria (NEJM 1996).